SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

PCV chemotherapy · 42-day cycle

NCCP 00379: adjuvant treatment of grade II glioma after radiotherapy, or palliative treatment of recurrent high-grade glioma. · NCCP 00379 · version 4 · 42-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
PCV combines procarbazine, lomustine and vincristine. In this 42-day regimen it may follow radiotherapy for grade II glioma or be used to help control recurrent high-grade glioma. Ask which setting and treatment aim apply to you.

Schedule
Lomustine once on Day 1; procarbazine daily on Days 8–21; vincristine into a vein on Days 8 and 29. The cycle lasts 42 days. The number of cycles is individual; treatment continues only while benefit and tolerance support it.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Lomustine
ORAL CHEMOTHERAPY CAPSULE

- ☐ Delayed and cumulative low blood counts
- ☐ Nausea / vomiting
- ☐ Lung injury
- ☐ Kidney and liver changes
- ☐ Rare second cancers and fertility effects

Procarbazine
ORAL CHEMOTHERAPY CAPSULE

- ☐ Low blood counts and infection
- ☐ Nausea / vomiting and appetite changes
- ☐ Drug, food and alcohol interactions
- ☐ Liver changes and rash
- ☐ Fertility effects and rare second cancers

Vincristine
CHEMOTHERAPY GIVEN INTO A VEIN

- ☐ Peripheral and autonomic nerve problems
- ☐ Constipation and bowel obstruction
- ☐ Jaw pain and weakness
- ☐ Tissue injury if medicine leaks outside the vein
- ☐ Hair thinning / loss

Discuss treatment intent, evidence and alternatives; phase-specific schedules; blood and organ monitoring; infection, nausea, fertility and pregnancy precautions; practical support and any unlicensed use. Keep the separate radiotherapy consent and medication plans where applicable. Explain the unlicensed oral medicine supply and any off-label indication, delayed/cumulative marrow effects, single-dose lomustine, IV-only vincristine, peripheral/autonomic neuropathy, lung risks, and procarbazine food/alcohol/medicine interactions.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00379 v4.

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